

hovid-Escitalopram Tablet

VIESC01-var6

DESCRIPTION

hovid-Escitalopram Tablet 10mg:
Oval, white film-coated tablet, shallow convex and scored on one face

hovid-Escitalopram Tablet 20mg:
Oval, white film-coated tablet, shallow convex with 'H', 'D' and break bar embossed on one face

COMPOSITION

hovid-Escitalopram Tablet 10mg:
Escitalopram oxalate 12.775mg equivalent to Escitalopram 10mg

hovid-Escitalopram Tablet 20mg:
Escitalopram oxalate 25.55mg equivalent to Escitalopram 20mg

PHARMACODYNAMICS

Escitalopram is a selective inhibitor of serotonin (5-HT) re-uptake with high affinity for the primary binding site. It also binds to an allosteric site on the serotonin transporter, with a 1000 fold lower affinity.

Escitalopram has no or low affinity for a number of receptors including 5-HT1A, 5-HT2, DAD1 and D2 receptors, α1-, α2-, β-adrenoceptors, histamine H1, muscarine cholinergic, benzodiazepine, and opioid receptors.

The inhibition of 5-HT re-uptake is the only likely mechanism of action explaining the pharmacological and clinical effects of escitalopram.

PHARMACOKINETICS

Absorption

Absorption is almost complete and independent of food intake. (Mean time to maximum concentration (mean T_{max}) is 4 hours after multiple dosing). As with racemic citalopram, the absolute bioavailability of escitalopram is expected to be about 80%.

Distribution

The apparent volume of distribution (Vd,β/F) after oral administration is about 12 to 26 L/kg. The plasma protein binding is below 80% for escitalopram and its main metabolites.

Metabolism

Escitalopram is metabolised in the liver to the demethylated and didemethylated metabolites. Both of these are pharmacologically active. Alternatively, the nitrogen may be oxidised to form the N-oxide metabolite.

Both parent substance and metabolites are partly excreted as glucuronides. After multiple dosing the mean concentrations of the demethyl and didemethyl metabolites are usually 28-31% and <5%, respectively, of the escitalopram concentration. Biotransformation of escitalopram to the demethylated metabolite is mediated primarily by CYP2C19. Some contribution by the enzymes CYP3A4 and CYP2D6 is possible.

Excretion

The elimination half-life (t_{1/2}) after multiple dosing is about 30 hours and the oral plasma clearance (CL_{oral}) is about 0.6 L/min. The major metabolites have a significantly longer half-life. Escitalopram and major metabolites are assumed to be eliminated by both the hepatic (metabolic) and the renal routes, with the major part of the dose excreted as metabolites in the urine.

Linearity

There is linear pharmacokinetics. Steady-state plasma levels are achieved in about 1 week. Average steady-state concentrations of 50 nmol/L (range 20 to 125 nmol/L) are achieved at a daily dose of 10 mg.

Elderly patients (>65 years)

Escitalopram appears to be eliminated more slowly in elderly patients compared to younger patients. Systemic exposure (AUC) is about 50 % higher in elderly compared to young healthy volunteers.

Reduced hepatic function

In patients with mild or moderate hepatic impairment (Child-Pugh Criteria A and B), the half-life of escitalopram was about twice as long and the exposure was

about 60% higher than in subjects with normal liver function.

Reduced renal function

With racemic citalopram, a longer half-life and a minor increase in exposure have been observed in patients with reduced kidney function (CL_{cr} 10-53 ml/min).

Polymorphism

It has been observed that poor metabolisers with respect to CYP2C19 have twice as high a plasma concentration of escitalopram as extensive metabolisers. No significant change in exposure was observed in poor metabolisers with respect to CYP2D6.

INDICATIONS

- Treatment of major depressive episodes.
- Treatment of panic disorder with or without agoraphobia.
- Treatment of social anxiety disorder (social phobia).
- Treatment of generalised anxiety disorder.
- Treatment of obsessive-compulsive disorder.

CONTRAINDICATIONS

- Hypersensitivity to the active substance or to any of the excipients.
- Concomitant treatment with non-selective, irreversible monoamine oxidase inhibitors (MAO-inhibitors) is contraindicated due to the risk of serotonin syndrome with agitation, tremor, hyperthermia etc.
- The combination of escitalopram with reversible MAO-A inhibitors (e.g. moclobemide) or the reversible non-selective MAO-inhibitor linezolid is contraindicated due to the risk of onset of a serotonin syndrome.
- Escitalopram is contraindicated in patients with known QT interval prolongation or congenital long QT syndrome.
- Escitalopram is contraindicated together with medicinal products that are known to prolong the QT interval.

WARNING AND PRECAUTIONS

Clinical Worsening / Suicide Risk

Monitor for clinical worsening, suicidality and unusual change in behaviour, especially during the initial few months of therapy or at times of dose changes.

Serotonin Syndrome

Serotonin syndrome has been reported with SSRI and SNRIs, including escitalopram, both when taken alone, but especially when co-administered with other serotonergic agents (including triptans, tricyclic antidepressants, fentanyl, lithium, tramadol, tryptophan, buspirone and St. Jones Wort). If such symptoms occur, discontinue escitalopram and initiate supportive treatment. If concomitant use of escitalopram with other serotonergic drugs is clinically warranted, patients should be made aware of a potential increased risk for serotonin syndrome, particularly during treatment initiation and dose increases.

Discontinuation of treatment with Escitalopram

A gradual reduction in dose rather than abrupt cessation is recommended whenever possible.

Seizures

Prescribe with care in patients with a history of seizure.

Activation of Mania / Hypomania

Use cautiously in patients with a history of mania.

Hyponatremia

Can occur in association with syndrome of inappropriate antidiuretic hormone secretion (SIADH).

Abnormal bleeding

Use caution in concomitant use with NSAIDs, aspirin, warfarin or other drugs that affect coagulation.

Interference with Cognitive and Motor Performance

Use with caution when operating machinery.

Angle Closure Glaucoma

Angle closure glaucoma has occurred in patients with untreated anatomically narrow angle and treated with antidepressants.

Use in patients with Concomitant Illness

Use with caution in patients with diseases or conditions that produce altered metabolism or hemodynamic responses.

Suicidality in Children and Adolescents

- Antidepressants increase the risk of suicidality in children and adolescents with major depressive disorder (MDD) and other psychiatric disorders.
- Anyone considering the use of an antidepressant in a child or adolescent for any clinical use must balance the risk of increased suicidality with the clinical need.
- Patients who are started on therapy should be observed closely for clinical worsening, suicidality or unusual changes in behaviour.
- Families and caregivers should be advised to closely observe the patient and to communicate with the prescriber.
- The indication(s) approved in paediatric for the particular drug should be clearly stated / included. Escitalopram is not approved for use in paediatric patients less than 12 years of age.

PREGNANCY AND LACTATION

Pregnancy: Escitalopram should not be used during pregnancy unless clearly needed and after careful consideration of the risk/ benefit ratio.

Observational data indicate an increased risk (less than 2-fold) of postpartum haemorrhage following SSRI/SNRI exposure within the month prior to birth.

Lactation: It is expected that escitalopram will be excreted into human milk. Consequently, breast-feeding is not recommended during treatment.

INTERACTION WITH OTHER MEDICAMENTS AND OTHER FORMS OF INTERACTION

Pharmacodynamic interactions

Contraindicated combinations:

Irreversible non-selective MAOIs

Escitalopram is contraindicated in combination with non-selective, irreversible MAOIs. Escitalopram may be started 14 days after discontinuing treatment with an irreversible MAOI. At least 7 days should elapse after discontinuing escitalopram treatment, before starting a non-selective, irreversible MAOI.

Reversible, selective MAO-A inhibitor (moclobemide)

Due to the risk of serotonin syndrome, the combination of escitalopram with a MAO-A inhibitor such as moclobemide is contraindicated. If the combination proves necessary, it should be started at the minimum recommended dosage and clinical monitoring should be reinforced.

Reversible, non-selective MAO-inhibitor (linezolid)

The antibiotic linezolid is a reversible non-selective MAO-inhibitor and should not be given to patients treated with escitalopram. If the combination proves necessary, it should be given with minimum dosages and under close clinical monitoring.

Irreversible, selective MAO-B inhibitor (selegiline)

In combination with selegiline (irreversible MAO-B inhibitor), caution is required due to the risk of developing serotonin syndrome. Selegiline doses up to 10 mg/day have been safely co-administered with racemic citalopram.

QT interval prolongation

Pharmacokinetic and pharmacodynamic studies of escitalopram combined with other medicinal products that prolong the QT interval have not been performed. An additive effect of escitalopram and these medicinal products cannot be excluded.



Therefore, co-administration of escitalopram with medicinal products that prolong the QT interval, such as Class IA and III antiarrhythmics, antipsychotics (e.g. phenothiazine derivatives, pimozide, haloperidol), tricyclic antidepressants, certain antimicrobial agents (e.g. sparfl oxacin, moxifloxacin, erythromycin IV, pentamidine, anti-malarial treatment particularly halofantrine), certain antihistamines (e.g. astemizole, mizolastine), is contraindicated.

Combinations requiring precautions for use:

- Serotonergic medicinal products (e.g. tramadol, sumatriptan and other triptans)
- Medicinal products lowering the seizure threshold (e.g. antidepressants (tricyclics, SSRIs), neuroleptics (phenothiazines, thioxanthenes and butyrephe nones), mefloquin, bupropion and tramadol
- Lithium, tryptophan
- St John's wort
- Haemorrhage
- Alcohol
- Medicinal products inducing hypokalaemia / hypomagnesaemia

Pharmacokinetic interactions

Influence of other medicinal products on escitalopram

The metabolism of escitalopram is mainly mediated by CYP2C19. CYP3A4 and CYP2D6 may also contribute to the metabolism although to a smaller extent. The metabolism of the major metabolite S-DCT (demethylated escitalopram) seems to be partly catalysed by CYP2D6.

Caution should be exercised when used concomitantly with CYP2C19 inhibitors (e.g. omeprazole, esomeprazole, fluvoxamine, lansoprazole, ticlopidine) or cimetidine. A reduction in the dose of escitalopram may be necessary based on monitoring of side-effects during concomitant treatment.

Effect of escitalopram on the pharmacokinetics of other medicinal products

Escitalopram is an inhibitor of the enzyme CYP2D6. Caution is recommended when escitalopram is co-administered with medicinal products that are mainly metabolised by this enzyme, and that have a narrow therapeutic index, e.g. flecainide, propafenone and metoprolol (when used in cardiac failure), or some CNS acting medicinal products that are mainly metabolised by CYP2D6, e.g. antidepressants such as desipramine, clomipramine and nortriptyline or antipsychotics like risperidone, thioridazine and haloperidol. Dosage adjustment may be warranted.

Co-administration with desipramine or metoprolol resulted in both cases in a twofold increase in the plasma levels of these two CYP2D6 substrates.

MAIN SIDE/ ADVERSE EFFECTS

Adverse reactions are most frequent during the first or second week of treatment and usually decrease in intensity and frequency with continued treatment.

Very common:
Headache, nausea

Common:
Decreased appetite, increased appetite, weight increased, anxiety, restlessness, abnormal dreams, insomnia, somnolence, dizziness, paraesthesia, tremor, sinusitis, yawning, diarrhoea, constipation, vomiting, dry mouth, increased sweating, arthralgia, myalgia, ejaculation disorder, impotence, fatigue, pyrexia.

Uncommon:
Decreased weight, bruxism, agitation, nervousness, panic attack, confusional state, taste disturbance, sleep disorder, syncope, mydriasis, visual disturbance, tinnitus, tachycardia, epistaxis, gastrointestinal haemorrhages (including rectal haemorrhage), urticarial, alopecia, rash, pruritus, metrorrhagia, menorrhagia, oedema.

QT interval prolongation

Cases of QT interval prolongation and ventricular arrhythmia including torsade de pointes have been reported, predominantly in patients of female gender, with hypokalaemia, or with pre-existing QT interval prolongation or other cardiac diseases.

Withdrawal symptoms

Discontinuation of SSRIs/SNRIs (particularly when abrupt) commonly leads to discontinuation symptoms. Dizziness, sensory disturbance (including paraesthesia and electric shock sensations), sleep disturbances (including insomnia and intense dreams), agitation or anxiety, nausea and/or vomiting, tremor, confusion, sweating, headache, diarrhoea, palpitations, emotional instability, irritability, and visual disturbances are the most commonly reported reactions. Generally these events are mild to moderate and are self-limiting, however, in some patients they may be severe and/or prolonged. It is therefore advised that when escitalopram treatment is no longer required, gradual discontinuation by dose tapering should be carried out.

EFFECTS ON ABILITY TO DRIVE

AND USE MACHINES

Patients should be cautioned about the potential risk of an influence on their ability to drive a car and operate machinery.

OVERDOSE

Symptoms:
Symptoms seen in reported overdose of escitalopram include symptoms mainly related to the central nervous system (ranging from dizziness, tremor, and agitation to rare cases of serotonin syndrome, convulsion, and coma), the gastrointestinal system (nausea/vomiting), and the cardiovascular system (hypotension, tachycardia, QT interval prolongation, and arrhythmia) and electrolyte/fluid balance conditions (hypokalaemia, hyponatraemia).

Treatment:

There is no specific antidote. Establish and maintain an airway, ensure adequate oxygenation and respiratory function. Gastric lavage and the use of activated charcoal should be considered. Gastric lavage should be carried out as soon as possible after oral ingestion. Cardiac and vital signs monitoring are recommended along with general symptomatic supportive measures.

ECG monitoring is advised in case of overdose in patients with congestive heart failure/bradyarrhythmias, in patients using concomitant medications that prolong the QT interval, or in patients with altered metabolism, e.g. liver impairment.

DOSAGE AND ADMINISTRATION

Oral, hovid-Escitalopram Tablet is administered as a single daily dose and may be taken with or without food.

Safety of daily doses above 20mg has not been demonstrated.

Major depressive episodes

Usual dosage is 10 mg once daily. Depending on individual patient response, the dose may be increased to 20 mg daily.

Usually 2-4 weeks are necessary to obtain antidepressant response. After the symptoms resolve, treatment for at least 6 months is required for consolidation of the response.

Panic Disorder with or without agoraphobia

An initial dose of 5 mg is recommended for the first week before increasing the dose to 10 mg daily. The dose may be further increased, up to a maximum of 20 mg daily, dependent on individual patient response.

Maximum effectiveness is reached after about 3 months. The treatment lasts several months.

Social anxiety disorder (social phobia)

Usual dosage is 10 mg once daily. Depending on individual patient

response, the dose may be increased to a maximum of 20 mg daily.

Usually 2-4 weeks are necessary to obtain symptom relief. Treatment for 3 months is recommended to consolidate response. Long-term treatment of responders for 6 months has been shown to prevent relapse and can be considered on an individual basis; treatment benefits should be re-evaluated at regular intervals.

Generalised anxiety disorder

Usual dosage is 10 mg once daily. Depending on individual patient response, the dose may be increased to a maximum of 20 mg daily.

Treatment for 3 months is recommended to consolidate response. Long-term treatment of responders for 6 months has been shown to prevent relapse and can be considered on an individual basis; treatment benefits should be re-evaluated at regular intervals.

Obsessive-compulsive disorder

Usual dosage is 10 mg once daily. Depending on individual patient response, the dose may be increased to 20 mg daily.

Long-term treatment of patients responding to a 16-week open treatment phase has been studied for at least 24 weeks in patients receiving 10 or 20 mg/day. As OCD is a chronic disease, patients should be treated for a sufficient period to ensure that they are symptom free. This period may be several months or even longer.

Elderly patients (>65 years of age)

Initial treatment with half the usually recommended dose and a lower maximum dose should be considered.

Children and adolescents (<18 years of age)

hovid-Escitalopram Tablets should not be used in the treatment of children and adolescents under the age of 18 years.

Reduced renal function

Dosage adjustment is not necessary in patients with mild or moderate renal impairment. Caution is advised in patients with severely reduced renal function (CL_{CR} less than 30 ml/min)

Reduced hepatic function

An initial dose of 5 mg daily for the first two weeks of treatment is recommended. Depending on individual patient response, the dose may be increased to 10mg daily.

Poor metabolisers of CYP2C19

For patients who are known to be poor metabolisers with respect to CYP2C19, an initial dose of 5mg daily during the first two weeks of treatment is recommended. Depending on individual patient response, the dose may be increased to 10mg daily.

Discontinuation symptoms

When stopping treatment with hovid-Escitalopram Tablet, the dose should be gradually reduced over a period of at least one to two weeks in order to avoid possible discontinuation symptoms.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage : Store below 30°C.

Presentation/packing:
Blister pack of 10's, 30's, 100's

Product Registration Holder:
Hovid Bhd.,
121, Jalan Tunku Abdul Rahman,
30010 Ipoh, Malaysia.

Manufactured by:
Hovid Bhd.,
Lot 56442, 7 ½ Miles, Jalan Ipoh /
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